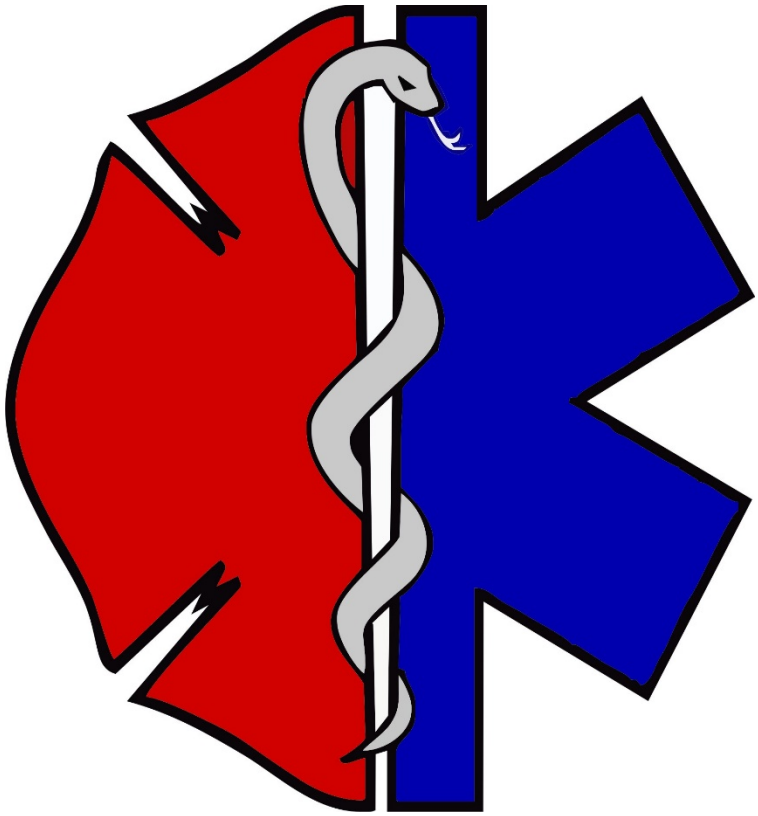
WISE COUNTY

FIRST RESPONDER PROTOCOLS



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GENERAL ORDERS FOR ALL PATIENTS

FRO personnel holding the highest level of EMS certification should be the responder to access and lead the treatment of the patient until Wise County EMS arrives. Once the transporting medic arrives the FRO personnel that is in charge at the scene will give a report to the transporting medic including patient findings and treatments that have occurred prior to arrival of Wise County EMS. Once the Wise County EMS medic makes patient contact the patient will be considered to be under the care of Wise County EMS.

When FRO Personnel arrive on scene:

1. Assess scene safety and patient condition.
2. Check responsiveness.
3. Check airway, breathing and circulation, (ABC's).
4. Control severe external bleeding.
5. Determine level of consciousness
6. Reassure the patient and keep him/her informed about treatment.
7. Obtain a brief history from the patient, family and/or bystanders.
8. Record vital signs and perform a physical examination. Check for Medical Alert tag.
9. Obtain list of medications and drug allergies, if applicable
10. Administer appropriate treatment per protocol in order of priority.
11. Fill out the First Responder report form.

CANCELLATION OF RESPONDING EMS UNIT

Canceling a responding EMS unit is allowable under the following conditions:

- Adult patients, who are conscious, fully oriented and who state they do not want to be transported to a hospital. (Patients with injuries have the right to refuse treatment/transport)
- An adult may refuse treatment/transportation for a minor child, if that adult is a family member or guardian, school personnel, daycare personnel or any adult with written permission to make healthcare decisions for that child
- Pregnant females of any age can refuse treatment/transport
- Any person who is enlisted in the U.S. Military, no matter their age, can refuse treatment/transport
- An adult who possess Durable Power of Attorney for Healthcare over another adult can refuse treatment/transport for the named adult, **UNLESS** the actual patient is Alert and Orientated and requesting treatment and or transport.
- Minors who do not live with an adult family member are considered to be an emancipated minor and can refuse treatment/transport.

If any of the above individuals state they do not want ambulance treatment/transportation, there is no need for EMS to continue solely for paperwork purposes. Have the patient sign the First Responder refusal form.

AIR EVACUATION CRITERIA

The following conditions are indications to be considered for air evacuation of a patient:

- Inaccessibility by ground ambulance due to terrain or environment conditions
- Level of care not available at local facility such as major burns, neurosurgical care
- Depleted ground transportation
- Penetrating trauma to the head, neck or chest
- Obvious spinal cord or closed head injuries
- Amputations which are candidates for re-attachment
- Priapism

ACCOMPANYING PATIENTS IN AMBULANCE

From time to time it may be necessary for FRO personnel to accompany patients in the ambulance. This may occur under the following conditions:

- There is a need for the FRO personnel to accompany the patient in the ambulance.
- The medic on the transporting ambulance approves.
- The FRO personnel will assist under direct supervision of the attending medic.
- The FRO personnel will wear seat belt while the ambulance in in motion.
- The FRO personnel will follow all HIPPA guidelines regarding patient confidentially.

ALLERGIC REACTION / ANAPHYLAXIS

1. Assess scene safety and patient condition
2. Establish and maintain a patent airway
3. Assist ventilation with BVM and supplemental oxygen if necessary
4. Provide oxygen via NRB mask at 12-15 lpm
5. Vital signs every 3-5 minutes
6. Treat patient for shock

EMT-B AND ABOVE

1. Assist patient with administration of Epinephrine Auto-Injector if prescribed to patient by a physician

AMPUTATIONS

ASSESSMENT CONSIDERATIONS

- Almost any part can be re-attached; arm, hand, fingers, leg, foot, scalp, penis, nose or ear
- Re-attachment can be performed as late as 24 hours after injury, but results are best when performed within a few hours of injury
- Speed is therefore important, but do not overlook other injuries

TREATMENT

1. Control bleeding, (see BLEEDING protocol)
2. Treat for shock, (see SHOCK protocol)
3. Care for amputated part
    - Rinse the part briefly with normal saline to remove loose debris; **do not scrub**
    - Wrap amputated part with sterile gauze moistened with normal saline
    - Place wrapped part into a plastic bag and seal **(do not pour more fluid into bag)**
    - Place plastic bag into cool container, **(do not place on ice)**
    - Transport part to hospital with the patient, if possible

BLEEDING

WITHOUT IMPALED OBJECT

1. Assess scene safety and patient condition.
2. Control bleeding:
   - Apply direct pressure and elevate the bleeding site if possible
   - If bleeding persists, place blood pressure cuff or tourniquet proximal to bleeding site. Inflate cuff to minimum pressure necessary to stop bleeding
3. Bandage wound and apply pressure as necessary to control bleeding
4. If bleeding is severe, treat for shock
5. Vital signs every 5 minutes

WITH IMPALED OBJECT

1. Assess scene safety and patient condition.
2. Apply direct pressure around impaled object
3. Immobilize the object. An impaled object in the cheek should only be removed if airway cannot be maintained
4. If bleeding persists, elevate the injured site
5. If bleeding continues, place a blood pressure cuff or tourniquet proximal to the injury. Inflate the cuff to the minimum pressure necessary to stop the bleeding
6. Treat for shock
7. Vital signs every 10 minutes

BREATHING DIFFICULTIES

1. Assess scene safety and patient condition
2. Ensure airway is patent
3. Assist ventilation with BVM and supplemental oxygen if necessary
4. Place patient in sitting position if possible
5. Provide oxygen via NRB mask at 12-15 lpm
6. Perform detailed history and physical exam. Vital signs every 3-5 minutes

EMT-B AND ABOVE

1. Assist patient with rescue inhaler or small volume nebulizer if prescribed to patient by a physician. Be sure to check expiration dates on medications

BURNS INJURIES

SOURCES OF BURNS

- Thermal (heat)
- Chemical
- Electrical

DEGREE OF BURN

FIRST-DEGREE (1°) – superficial injury involving only the epidermis of the skin. It is characterized by reddening of the skin and perhaps some swelling. First-degree burns are evaluated as partial-thickness burns.

SECOND-DEGREE (2°) – the first layer of skin is burned through and the second layer is damaged but the burn does not extend to underlying tissues. Second-degree burns are characterized by red, mottled skin with blisters. Burn will be accompanied by intense pain. These burns are also evaluated as partial-thickness burns.

THIRD-DEGREE (3°) – this is a full-thickness burn involving all layers of the skin. Areas of charred black or dry and white skin characterize these burns. There may be an absence of pain in these areas, but extreme pain on the periphery. Third-degree burns may be difficult to distinguish from second-degree burns.

SEVERITY OF BURNS

MINOR BURNS

- Third-degree burns involving less than 2% BSA, excluding face, hands, feet, groin or major joints
- Second-degree burns involving less than 15% BSA
- First-degree burns involving less than 20% BSA

MODERATE BURNS

- Third-degree burns that involve 2% to 10% BSA, excluding face, hands, feet, groin or major joints
- Second-degree burns that involve 15% to 30% BSA
- First-degree burns that involve 20% to 75% BSA

CRITICAL BURNS

- All burns complicated by other injuries, especially to the respiratory tract
- Second- or third-degree burns to the face, hands, feet, groin or major joints
- Third-degree burns involving more than 10% BSA
- Second-degree burns involving more than 30% BSA
- First-degree burns involving more than 75% BSA

USE THE "RULE OF NINES" (Attachment 1) TO DETERMINE % BODY SURFACE AREA (BSA) AFFECTED

THE EXTENT OF INJURY TO ALL BURN PATIENTS SHOULD BE DETERMINED ACCORDING TO THE ABOVE CRITERIA AND THAT INFORMATION REPORTED TO THE RESPONDING EMS UNIT.

TREATMENT OF BURN

THERMAL BURNS

1. Assess scene safety.
2. Stop the burning process. Wrap the victim in a fire blanket or roll him/her on the ground. If that is not possible, extinguish the flames with a multi-purpose dry chemical (ABC) fire extinguisher.
3. Perform initial patient assessment.
4. Administer oxygen via NRB mask at 12-15 lpm.
5. Remove all jewelry and non-adhering clothing from burned areas.
6. Performed detailed assessment to include complete burn classification, secondary injuries and vital signs every 3-5 minutes.
7. Dress burns with dry sterile dressings.
8. Treat secondary injuries appropriately.
9. Keep patient warm as severe burns can affect the body's temperature regulation.

CHEMICAL BURNS

1. Assess scene safety.
2. Make every attempt to determine the chemical or chemicals involved and take the appropriate personal protective measures.
3. Flush the affected areas with a copious and gentle flow of water. Except in the case of dry lime exposures- first, brush the dry chemical from the skin then begin to flush. Continue flushing for at least 20 minutes while removing clothing and jewelry.
4. Perform initial assessment.
5. Do not use antidotes.
6. Treat patient as directed by Thermal Burns protocol.

ELECTRICAL BURNS

ASSESSMENT CONSIDERATIONS

- Patient will have at least 2 burn sites, contact point with the source and contact point with a ground
- Respiratory distress or arrest are possible
- Irregular heart rate or complete cardiac arrest are possible and may be initial or delayed
- Fractured bones or dislocations from severe muscle contractions or fall
- Seizures
- Paralysis resulting from disrupted nerve pathways

TREATMENT

1. Assess scene safety
2. Ensure patient is safely removed from contact with electrical source and extinguish any active burning process
3. Perform initial assessment and intervene as required
4. Provide oxygen as indicated
5. Perform detailed assessment to include locating and characterizing burn sites, secondary MOIs (such as a fall), and locate any secondary injuries
6. Vital signs every 3-5 minutes.
7. Dress burn sites with dry sterile dressings, treat any secondary injuries appropriately
8. Treat patient for shock

CHEST INJURIES

SIMPLE RIB FRACTURES

1. Assess scene safety and patient condition.
2. Provide oxygen via NRB mask at 12-15 LPM
3. Vital signs every 10 minutes

FLAIL CHEST

1. Assess scene safety and patient condition.
2. Stabilize flail segment with bulky pad and cravats, do not tape
3. Provide oxygen via NRB mask at 12-15 LPM. Assist ventilations with BVM if necessary
4. Vital signs every 5 minutes
5. Place patient on injured side as tolerated by comfort of patient and ease of breathing

SUCKING CHEST WOUND

1. Assess scene safety and patient condition.
2. Cover the wound with occlusive dressing and tape on three sides only
3. Provide oxygen via NRB mask at 12-15 LPM. Assist ventilations with BVM if necessary
4. Vital signs every 5 minutes
5. Place patient on injured side as tolerated
6. If patient has increased difficulty breathing, uncover the wound immediately

CHEST PAIN

ASSESSMENT CONSIDERATIONS

- Chest pain may be a result of Angina, Acute Myocardial Infarction (AMI) or stress/anxiety reactions. Treat all patients complaining of chest pain as if they are having an AMI.
- Use the OPQRST method to assess chest pain.
- Signs and symptoms may include feeling of impending doom, SOB, restlessness, anxiety, pallor, diaphoresis, chest pain not alleviated by rest and or nitroglycerin
- Patient's vital signs may be in any range from below normal to normal to above normal
- 15% to 20% of patients having an AMI will have no pain at all or they may have atypical pain such as in the back or abdomen

TREATMENT

1. Assess scene safety
2. Perform initial assessment, intervene as required. Note the general appearance of the patient
3. Loosen any restrictive clothing, place patient in a position of comfort, reassure patient and keep him/her still calm and quiet
4. Provide oxygen via NRB mask at 12-15 lpm
5. Perform detailed physical exam and take as thorough a history as possible
6. Vital signs every 3-5 minutes

EMT-B AND ABOVE

1. Assist patient with their nitroglycerin unless patient's systolic blood pressure is below 100mm Hg. Total dosage not to exceed 3 doses. Be sure to check expiration dates on all medications
2. Give patient 2, 81mg chewable Aspirin

CHILDBIRTH

1. Assess scene safety and patient condition.
2. Check the mother for seizures and/or syncope
3. Vital signs every 10 minutes
4. Determine history
   - Age
   - Last menstrual period
   - Estimated date of confinement
   - Multiple gestation
   - Gravidity
   - Parity
   - Abortions
   - Presence of/absence of contractions, vaginal bleeding or watery discharge, feeling of needing to move bowels
5. Perform physical exam. Gently palpate the mother's abdomen. Inspect the perineum for crowning with the next contraction
6. Prepare for imminent delivery if crowning is noted

NORMAL VERTEX DELIVERY

7. As the baby's head becomes visible, use the palm of your hand to apply a GENTLE back pressure to prevent an explosive delivery and minimize tearing of the perineum
8. When the head is delivered, use a bulb syringe to suction the mouth and then the nose. Support the head with both hands.
9. Once the head is delivered, the baby will rotate to the left or right. To deliver the anterior shoulder, apply GENTLE, downward traction to the head.
10. The posterior shoulder will deliver next. Once the shoulders have delivered, the rest of the baby will deliver almost explosively, so be prepared.
11. Keep the baby at the same level as the vagina until the umbilical cord is clamped to avoid autotransfusion.
12. Place an umbilical clamp 7-10 inches from the baby. Place a second clamp between the baby and the first clamp, about 2-3 inches from the first.
13. Cut the cord BETWEEN the two clamps with STERILE scissors or scalpel
14. Dry the baby with clean towels, suction the airway as needed and place the baby on the mother's abdomen wrapped with an infant thermal blanket or clean dry towel. (A new born infant can become hypothermic very easily if not kept warm. Turn off ceiling fans and A/C if needed.)
15. Perform an APGAR assessment of the baby at 1 and 5 minutes after delivery.
16. Prepare for delivery of the placenta. Massage the mother's uterus in a circular motion to stimulate contractions. NEVER pull on the umbilical cord to speed delivery of the placenta.
17. Once the placenta has delivered, examine it to determine if it is intact, then place it in a basin or bag for transport with the mother and baby.
18. Reassess the mother and baby continually until EMS arrives.

CHILDBIRTH (cont.)

COMPLICATIONS OF CHILDBIRTH

MEMBRANES HAVE NOT RUPTURED

1. Use your fingers to rupture the membranes
2. Pull the membranes away from the baby's face

CORD IS WRAPPED AROUND THE BABY'S NECK

1. Try to place two fingers under the cord at the back of the baby's neck and loosen the cord
2. Slip the cord over the baby's head
3. If the cord cannot be loosened, place two clamps on the cord where possible
4. Carefully cut the cord between the clamps and unwind the cord from the baby's neck

PROLAPSED CORD

1. Place the mother in a face down, knees-to –chest position, or keep the mother supine and place pillows or blankets under the mothers buttocks
2. Provide the mother with oxygen via NRB mask at 12-15 LPM
3. Wrap the cord with sterile, moist dressings and try to keep the cord warm
4. Insert the fingers of a gloved hand into the vagina and lift any presenting part of the baby off of the umbilical cord.
5. Be prepared to remain in this position until patient is delivered to surgery

VAGINAL BLEEDING

1. Treat the patient for shock
2. Place absorbent pads or dressings at vaginal opening. DO NOT pack the vagina
3. Massage the uterus in a circular motion to promote contraction of the uterus

COLD EMERGENCIES

FROSTBITE

1. Assess scene safety and patient condition.
2. Vital signs every 10 minutes
3. Classify frostbite: 1° - erythema and edema without blister formation
   2° - erythema and edema with large, clear blisters
   3° - erythema and edema with hemorrhagic vesicles
4. Remove all clothing and socks from the affected area
5. Immerse the affected area in lukewarm water (100-105° F) until resolution of numbness
6. Cover the re-warmed area with dry sterile dressing
7. Elevate and immobilize the affected area

SYSTEMIC HYPOTHERMIA

1. Assess scene safety and patient condition, then contact responding EMS unit
2. Remove all wet clothing and wrap patient in dry blankets
3. Vital signs every 5 minutes
4. Place patient in a warm environment
5. If ETA of EMS unit is less than 30 minutes, do not attempt to re-warm patient
6. If ETA of EMS unit is greater than 30 minutes, apply re-warming blanket. If a re-warming blanket is not available, place hot packs around the patient's head and neck and in the axillary and groin areas.

CPR - AED

(Adult and Pediatric)

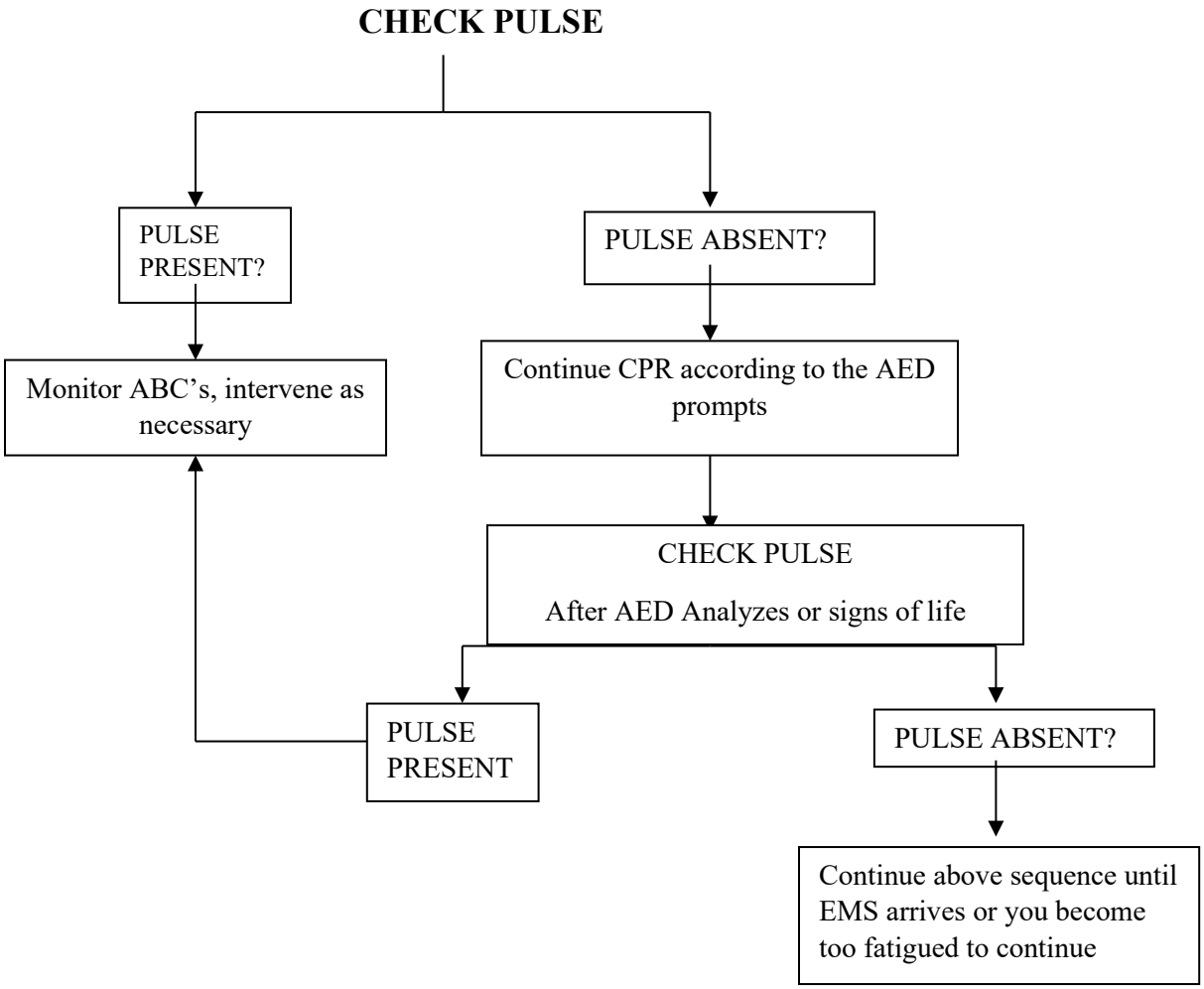
INDICATIONS

- Patients who are unresponsive and pulseless and apneic
- If pediatric patient (<8yo or <90lbs) use pediatric pads

CONTRAINDICATIONS

- Patients who are conscious or have a pulse or spontaneous respirations

1. Assess scene safety
2. Perform initial assessment
3. Initiate CPR until AED is available and attached
4. Stop CPR and press the ANALYZE button
5. Follow AED prompts.



CEREBRAL VASCULAR ACCIDENT (CVA)

TRANSIENT ISCHEMIC ATTACK (TIA)

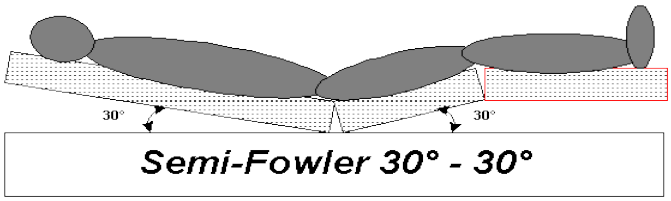
(STROKE)

ASSESSMENT CONSIDERATIONS

- Patients suffering from TIAs are at risk to suffer a stroke, even though patient may have complete resolution of symptoms
- Perform a thorough history and complete neurological exam
- A conscious patient suffering from a CVA can deteriorate quickly so be prepared to intervene

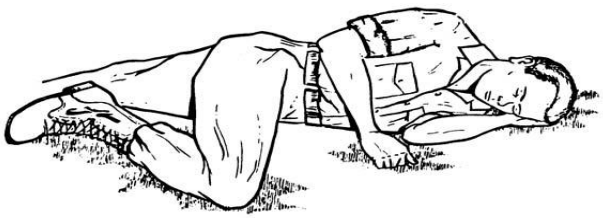
TREATMENT

1. Assess scene safety
2. Perform initial assessment, be prepared to suction airway or position patient to prevent aspiration
3. Provide oxygen via NRB mask at 12-15 lpm
4. If patient is unconscious, see below; if patient is conscious , perform detailed history and physical exam including neurological exam
5. Keep patient calm and reassured
6. Vital signs every 3-5 minutes
7. Place patient in Semi-Fowler's position



Unconscious patient

1. Maintain patent airway and ventilate patient with BVM and supplemental oxygen if indicated
2. Place patient in lateral recumbent position



DIABETIC EMERGENCIES

CONSCIOUS PATIENT

1. Assess scene safety and patient condition.
2. patient is alert, has a gag reflex and is able to drink without assistance, have the patient eat or drink a substance containing sugar (orange juice with 2-3 tablespoons of sugar mixed in it)
3. Vital signs every 5-10 minutes

EMT-B AND ABOVE

If patient is alert, has a gag reflex and able to swallow, provide oral glucose.

UNCONSCIOUS PATIENT

1. Assess scene safety and patient condition, then contact responding EMS unit
2. Prevent aspiration
   - Suction secretions
   - Place patient on his side in recovery position

The recovery position



✓Chin is well up to keep the respiratory passage open
✓Mouth is downward to enable drainage
✓Arms and legs are locked for stability

3. Administer high-flow oxygen (10-12 LPM via NRB mask)

EMOTIONALLY DISTURBED PATIENTS

1. Assess scene safety and patient condition.
2. Treat life-threatening injuries
3. Assess the situation and call for police support. If the patient appears dangerous, has a lethal weapon or seems in danger of losing control, or had violent, disruptive or self-destructive impulses, the law-enforcement personnel may aid in providing the necessary physical restraints
4. Approach the patient in a direct, honest manner. Maintain continuous contact with the patient. Encourage the patient to discuss situational stresses and be alert for emotional instability such as mood swings, paranoid delusions and depression
5. Avoid restraining the patient, if possible, but once restraints are applied do not remove them
6. Treat non-life-threatening injuries as the patient allows

EVISCERATION

1. Assess scene safety and patient condition.
2. Control bleeding
3. Treat for shock
4. Provide oxygen via NRB mask at 12-15 LPM
5. Cover eviscerated tissue with moist, sterile dressings and place an occlusive cover over that dressing
6. **DO NOT** attempt to replace the eviscerated organs back into the abdominal cavity
7. Vital signs every 5 minutes

FRACTURES AND DISLOCATIONS

1. Assess scene safety and patient condition.
2. Control bleeding
3. Check pulse and sensation distal to injury
4. If a distal pulse is present, immobilize the fracture/dislocation as it is found.
5. If distal pulse is absent, apply gentle traction to the extremity while repositioning it until pulse returns. If pulse does not return, immobilize the extremity
6. Vital signs every 10 minutes
7. Treat for shock if indicated

HEAD/NECK/SPINE INJURIES

1. Assess scene safety and perform initial patient assessment, then contact responding EMS unit.
2. Establish and maintain manual stabilization of cervical spine.
3. **DO NOT** hyperextend the patient's neck. Use the modified jaw thrust maneuver to open the airway, if necessary.
4. If not breathing, ventilate the patient with a BVM at rate of 24 breaths/min with supplemental oxygen, (10-12 lpm). If patient is conscious, administer oxygen via non-rebreather mask at 12-15 lpm.
5. Control obvious external bleeding (except from ears or a possible depressed skull fracture).
6. Perform a neurological examination (level of consciousness, posture, sensation, movement and pupils) before and after immobilization.
7. Securely immobilize the patient to a long spine board.
8. Obtain vital signs every 5 minutes.
9. If the patient shows signs and symptoms of shock, elevate the foot of the long spine board.
10. Search for and treat other injuries.
11. Prevent aspiration:
    a. Suction secretions from mouth as needed
    b. If vomiting occurs, turn patient on his/her side as a unit.

HEAT EMERGENCIES

HEAT EXHAUSTION
(Cool, clammy skin)

1. Assess scene safety and patient condition.
2. Remove patient to a cool environment
3. Place patient in supine position
4. Vital signs every 10 minutes

HEAT STROKE
(Core temperature greater than 104° F, hot/dry skin, altered level of consciousness)

1. Assess scene safety and patient condition, then contact responding EMS unit
2. Remove patient to a cool environment
3. Reduce body temperature immediately. Place ice packs around the patient's head and neck, and in the axillary and groin areas. If ice is not available, wrap the patient in a sheet and wet the sheet with water.
4. Stop the cooling process if patient starts to shiver
5. Vital signs every 5 minutes
6. Administer high-flow O2 (10-12 LPM) via NRB mask

MULTIPLE TRAUMA

Assess scene safety and patient condition.

1. Do not hyperextend the neck. Use the modified jaw thrust to open airway
2. If not breathing, ventilate the patient 24 times/minute with BVM and supplemental oxygen
3. Search for and treat sucking chest wounds
4. Control severe bleeding
5. Maintain gentle traction on head until head and neck are immobilized
6. Perform a brief physical examination
7. Treat for shock

POISONING

INGESTED

1. Assess scene safety and patient condition.
2. Determine the nature of the poisoning; What, When and How much was taken (Relay information to responding EMS as soon as possible)
3. Perform a brief physical assessment
4. Vital signs every 5 minutes
5. Transport substance container and any emesis to hospital with patient
6. Place the patient on his/her side
7. Treat other problems/injuries as needed

INHALED

1. **Assess scene safety** and patient condition, then contact responding EMS unit
2. Remove patient from poisonous atmosphere
3. Provide oxygen via NRB mask at 12-15 LPM
4. Vital signs every 5 minutes

INJECTED (Bites and Stings)

1. Assess scene safety and patient condition, then contact responding EMS unit
2. Keep the victim quiet
3. Remove all jewelry and tight fitting clothing from the affected limb
4. Immobilize the affected limb
5. Vital signs every 10 minutes
6. Be alert for S/S of anaphylaxis

RADIATION EXPOSURE

The vast majority of accidents involving radioactive materials occur in facilities, which use these materials daily. In these circumstances, rescue squads should seek and follow the professional advice that is readily available to them in these facilities. For accidents involving radioactive materials where professional guidance is unavailable, the following guidelines should be followed.

1. Assess scene safety and patient condition then contact responding EMS unit, who is expected to coordinate care with law enforcement, fire and state agencies

2. If victims without serious injuries are involved:
   - Do not enter the area suspected of having radioactive material present
   - Do not permit spectators to enter area
   - Do not allow victims to leave the area

3. If victims with serious injury are involved in the accident:
   - Treat life-threatening injuries
   - Remove the patient from the hazard area as soon as possible
   - Obtain vital signs every 10 minutes
   - Search for and treat other injuries
   - Remove contaminated outer clothing and wash all exposed skin
   - Wrap patient in a blanket

SEIZURES

1. Assess scene safety
2. If patient is actively convulsing, place the patient on the floor or ground, do not attempt to restrain the patient but protect the patient from harm. **NEVER** attempt to force any object into the mouth of a seizure patient
3. Be prepared to suction the patient's airway or position the patient to prevent aspiration
4. Once convulsions have ceased, perform initial assessment and intervene as necessary
5. Provide oxygen via NRB mask at 12-15 lpm
6. Perform detailed history and physical exam. Vital signs every 5 minutes. Treat any secondary injuries as appropriate

SEXUAL ASSAULT

1. Assess scene safety and patient condition.
2. Treat life-threatening injuries
3. Maintain as much privacy for the patient as possible. If possible have same sex responder treat the patient.
4. Offer emotional support. Concentrate history on medical aspects of the case
5. Search for and treat other injuries. If possible, do not disturb the scene of the assault or remove any clothing
6. When contacting the police and the responding EMS unit, do not identify the victim by name.
7. Do not allow the patient to shower, bathe, douche, urinate or defecate
8. Do not allow the patient to change clothes before being transported to the hospital by EMS
9. Arrangements may need to be made for additional clothing the patient can wear home from the hospital

SHOCK

Hemorrhagic Shock

1. Assess scene safety and patient condition.
2. Control external bleeding (see BLEEDING protocol)
3. Administer high-flow O2 (10-12 LPM) via NRB mask
4. Splint fractures
5. Elevate the lower extremities
6. Place blankets under and over patient
7. Vital signs every 5 minutes

Cardiogenic Shock

1. Assess scene safety and patient condition.
2. Administer high-flow O2 (10-12 LPM) via NRB mask
3. Place patient in semi-Fowler's position. If patient is hypotensive, elevate lower extremities
4. Place blankets under and over patient
5. Vital signs every 5 minutes

SNAKEBITE

1. Assess scene safety and patient condition.
2. Keep the victim still and quiet
3. Remove all jewelry and tight clothing from the affected extremity
4. Keep the affected extremity at or below the level of the heart
5. Vital signs every 5 minutes
6. Immobilize the affected extremity
7. **DO NOT APPLY TURNIQUET**
8. **DO NOT APPLY ICE PACK**

If possible identify the type of snake with either a picture or if the snake is dead it can be placed in an enclosed container and transported.

VAGINAL BLEEDING

1. Assess scene safety and patient condition.
2. Place dressing or pad beneath the patient
3. Vital signs every 5 minutes
4. Determine history:
   - Last menstrual period
   - Contraception
   - Sexual history
5. Treat for shock
6. Save any tissues passed by the patient

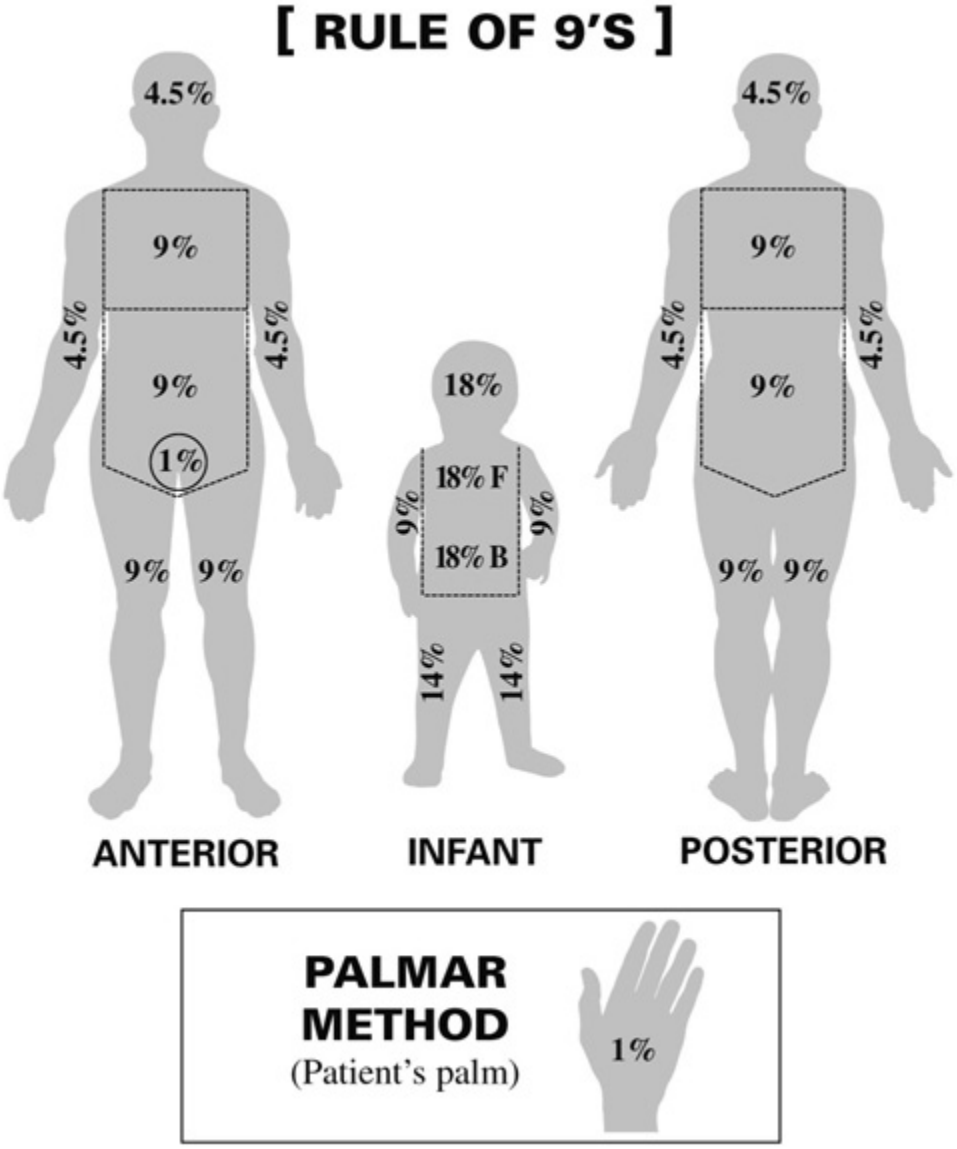
GLASGOW COMA SCALE

	VICTIM'S RESPONSE	DEFINITIONS	ASSIGNED SCORE
EYE OPENING	Spontaneous	Opens eyes without any stimulation	4
	To voice	**Opens eyes in response to command Shout the command if no response**	**3**
	To pain	Opens eyes in response to a brisk sternal Rub if voice command fails	2
	None	Does not open eyes to any stimulus	1
VERBAL RESPONSE	Oriented	Patient is awake and oriented to Person Place and Time	5
	Confused	Cannot give accurate responses, but is Able to use phrases or sentences and Perhaps carry on a conversation	4
	Inappropriate words	Patient may say one or more inappropriate Words, call out for specific people	3
	Incomprehensible words	Patient mumbles, groans or moans in Response to stimulus	2
	None	Repeated stimulation provokes no response From patient	1
MOTOR RESPONSE	Obeys commands	Patient performs simple motor task in Response to request	6
	Localizes pain	Patient reaches for the source of pain May try to remove your hand from pain Site	5
	Withdraws (pain)	Patient tries to move away from source of Pain	4
	Flexion (pain)	Patient will flex extremities in response to Pain	3
	Extension (pain)	Patient will extend extremities in response To pain, shoulder and forearm may rotate Internally	2
	None	Patient does not respond to repeated Application of stimulus	1

MAXIMUM SCORE = 15

MINIMUM SCORE = 3

* A BRISK STERNAL RUB IS THE PREFERRED METHOD OF PRODUCING A PAINFUL STIMULUS
** PATIENTS MAY HAVE EXISTING CONDITIONS WHICH WILL COMPLICATE OR MAKE IT IMPOSSIBLE TO COMPLETE A GLASGOW COMA SCORE


[RULE OF 9'S]
4.5%
9%
4.5%
9%
4.5%
1%
9%
9%
ANTERIOR
18%
18% F
9%
9%
18% B
14%
14%
INFANT
4.5%
9%
4.5%
9%
4.5%
9%
9%
POSTERIOR
PALMAR METHOD
(Patient's palm)
1%

Appendix 2

APGAR SCORING SYSTEM

	0 Points	1 Point	2 Points	Points totaled
Activity (muscle tone)	Absent	Arms and legs flexed	Active movement	
Pulse	Absent	Below 100 bpm	Over 100 bpm	
Grimace (reflex irritability)	Flaccid	Some flexion of Extremities	Active motion (sneeze, cough, pull away)	
Appearance (skin color)	Blue, pale	Body pink, Extremities blue	Completely pink	
Respiration	Absent	Slow, irregular	Vigorous cry	

Severely depressed	0-3
Moderately depressed	4-6
Excellent condition	7-10

Patient Assessment Chart

SAMPLE

- **S**igns/symptoms
- **A**llergies
- **M**edication
- **P**ast pertinent history
- **L**ast oral intake
- **E**vents leading to injury

OPQRST

- **O**nset
- **P**rovocation
- **Q**uality
- **R**adiation
- **S**everity
- **T**ime

DCAP-BTLS

- **D**eformities
- **C**ontusions
- **A**brasions
- **P**unctures
- **B**urns
- **T**enderness
- **L**acerations
- **S**welling

Baseline Vitals

- ✓ **Breathing**
- ✓ **Pulse**
- ✓ **Skin**
- ✓ **Pupils**
- ✓ **Blood pressure**
- ✓ **Pulse oximetry**

Appendix 3